

Global tuberculosis report 2024

TB SITUATION AND RESPONSE

Tuberculosis (TB) is contagious and airborne.

In 2023, TB returned to being the world's leading infectious disease killer, surpassing COVID-19. It was also the leading killer of people with HIV and a major cause of deaths related to antimicrobial resistance.

TB BURDEN

- In 2023, an estimated 10.8 million (95% uncertainty interval [UI]: 10.1–11.7 million) people fell ill with TB worldwide, of which 55% were men, 33% were women and 12% were children and young adolescents. People living with HIV accounted for 6.1% of the total.
- The TB incidence rate (new cases per 100 000 population per year) is estimated to have increased by 4.6% between 2020 and 2023, reversing declines of about 2% per year between 2010 and 2020.
- Globally in 2023, TB caused an estimated 1.25 million (95% UI: 1.13–1.37 million) deaths, including 161 000 people with HIV. This was down from best estimates of 1.32 million (95% UI: 1.21–1.45 million) in 2022.
- Eight countries accounted for more than two thirds of the global total: India, Indonesia, China, the Philippines, Pakistan, Nigeria, Bangladesh and the Democratic Republic of the Congo. The top five countries accounted for 56% of the global total.

TB CARE AND TREATMENT

- Global efforts to combat TB have saved an estimated 79 million lives since 2000.
- Globally in 2023, the reported number of people newly diagnosed with TB was 8.2 million, up from 7.5 million in 2022 and far above the levels of 5.8 million in 2020 and 6.4 million in 2021. Those newly diagnosed in 2022 and 2023 probably included a sizeable backlog of people who developed TB in previous years, but whose diagnosis and treatment was delayed by COVID-related disruptions.
- There is still a large global gap between the estimated number of people who fell ill with TB and the number of people newly diagnosed, with approximately 2.7 million people not diagnosed with the disease, or not officially reported to national authorities in 2023, down from around 4 million in both 2020 and 2021 and below the pre-pandemic level of 3.2 million in 2019.

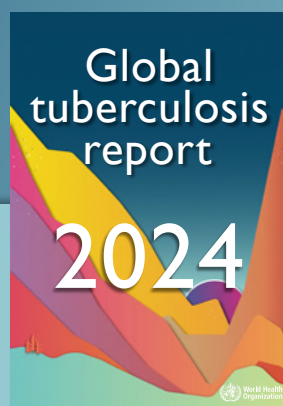
DRUG-RESISTANT TB

- Globally, an estimated 400 000 people (95% UI: 360 000–440 000) developed multidrug-resistant or rifampicin-resistant TB (MDR/RR-TB) in 2023.
- The number of people diagnosed with MDR/RR-TB and started on treatment was 175 923 people in 2023, a slight fall (of 1.1%) from 177 912 in 2022 and below the pre-pandemic level of 181 533 in 2019.
- In 2023, treatment success rates for drug-susceptible TB sustained at 88% and for MDR/RR-TB was improved to 68%.

ADDRESSING THE CO-EPIDEMICS OF TB AND HIV

- Among all incident cases of TB in 2023, 662 000 people living with HIV fell ill with TB, with the highest burden in countries in the WHO African Region.
- The global coverage of HIV testing among people diagnosed with TB re-mained high in 2023, at 80%. This was the same level as in 2022, but up from 76% in 2021 and 73% in 2020.

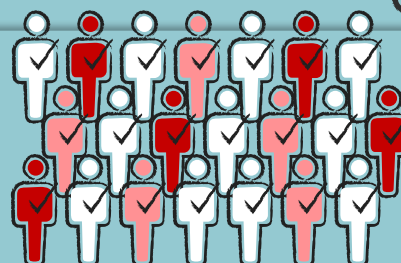
TUBERCULOSIS IS THE WORLD'S LEADING INFECTIOUS DISEASE KILLER



1.25 MILLION
TB DEATHS INCLUDING
161 000
DEATHS AMONG
PEOPLE WITH HIV

TB is also the leading cause of deaths among people with HIV and a major contributor to deaths associated with antimicrobial resistance.

IN 2023 AN ESTIMATED
10.8 MILLION
PEOPLE FELL ILL WITH TB



79 MILLION
LIVES SAVED SINCE THE
YEAR 2000 DUE TO GLOBAL
EFFORTS TO COMBAT TB

**DRUG-RESISTANT TB
REMAINS A PUBLIC
HEALTH CRISIS**
WITH GAPS IN DETECTION
AND TREATMENT.

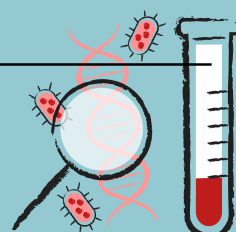
Only about
2 IN 5 PEOPLE
with drug-resistant TB
accessed treatment



US\$ 22 BILLION
REQUIRED PER YEAR FOR
TB PREVENTION, DIAGNOSIS,
TREATMENT AND CARE

US\$ 5.7 BILLION
WAS AVAILABLE IN 2023
of which 80% domestic
financing and US\$ 1.2 billion
international financing

**US\$ 5
BILLION**
REQUIRED PER
YEAR FOR TB
RESEARCH



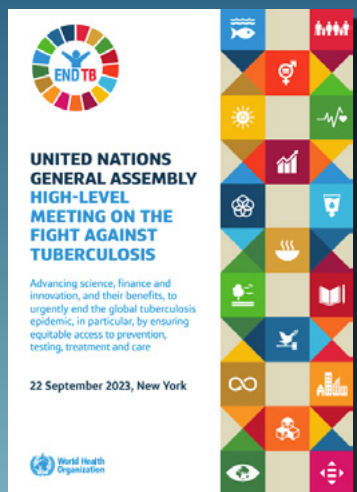
**US\$ 4
BILLION**
FUNDING GAP

TB PREVENTIVE TREATMENT

- WHO recommends TB preventive treatment for people living with HIV, household contacts of those with bacteriologically confirmed pulmonary TB, and clinical risk groups (e.g. those receiving dialysis).
- Globally in 2023, TB preventive treatment was provided to 4.7 million people.
- Since 2021, there has been a particularly noticeable increase in the number of household contacts enrolled on TB preventive treatment: from 0.76 million in 2021 to 2.7 million in 2023. In contrast, the number of people living with HIV who were enrolled on TB preventive treatment increased between 2015 and 2019 (reaching a peak of 3.0 million in 2019) before falling in 2020 and subsequently levelling off at about 2 million people per year.
- The estimated coverage of TB preventive treatment among household contacts reached 21% in 2023, representing substantial progress from 2015 (<1%) and 2019 (5.0%). For people living with HIV, coverage among those newly enrolled on ART was higher, at 56% in 2023; however, this was down from a peak of 68% in 2019. The global target of 90% coverage by 2027 remains some way off.

UPTAKE OF DIAGNOSTICS, NEW DRUGS AND REGIMENS

- Increasing access to early and accurate diagnosis using a molecular WHO-recommended rapid diagnostic test is one of the main components of TB laboratory-strengthening efforts under the End TB Strategy.
- The use of rapid diagnostic test remains far too limited. A WHO-recommended rapid molecular test was used as the initial diagnostic test for only 48% of the 8.2 million people newly diagnosed with TB in 2023, up from 47% in 2022 and 38% in 2021.
- By the end of 2023, 58 countries had started to use the all-oral 6-month BPaLM/BPaL regimen to treat people with MDR/RR-TB or pre-XDR-TB, up from 41 by the end of 2022. A total of 100 countries were using the all-oral 9-months regimens for the treatment of MDR/RR-TB, up from 95 in 2022 and 93 in 2021.
- There was an increase in access to shorter (1–3 months) rifamycin-based regimens for TB preventive treatment. In 2023, it reached 1.0 million people in 86 countries in 2023, up from 0.60 million people in 74 countries in 2022.



The second UN high-level meeting on TB was held on 22 September 2023. The resulting political declaration reaffirms existing commitments and targets and includes new ones for the period 2023–2027

[Read more](#)

RESEARCH AND INNOVATION

- The diagnostic pipeline has expanded considerably in terms of the number of diagnostic classes, tests, products and methods in development. There are new biomarker-based point-of-care and near point-of-care test classes for the diagnosis of TB disease, and new tests in existing diagnostic classes.
- There were 15 vaccine candidates in clinical trials by August 2024: four in Phase I, five in Phase II and six in Phase III. They included candidates to prevent TB infection and TB disease, and to help improve the outcomes of treatment for TB disease.
- In August 2024, there were 29 drugs for the treatment of TB disease in Phase I, Phase II or Phase III trials. This is an increase from eight in 2015.
- There are at least 30 clinical trials and implementation research studies underway to evaluate drug regimens and models of delivery for TB preventive treatment.

UNIVERSAL HEALTH COVERAGE, SOCIAL PROTECTION, SOCIAL DETERMINANTS AND MULTISECTORAL ACTION

- Global TB targets for reductions in TB disease burden can only be achieved if TB diagnostic, treatment and prevention services are provided within the context of progress towards UHC, and if there is multisectoral action to address the broader determinants that influence TB epidemics.
- About 50% of people treated for TB and their households face total costs (direct medical expenditures, direct non-medical expenditures and indirect costs such as income losses) that are catastrophic (>20% of annual household income), far from the WHO End TB Strategy target of zero.
- In 2024, WHO requested all countries to report data about national policies related to financial and social protection for the first time, as a starting point for assessment of the status of progress with respect to a new target of the second UN high-level meeting on TB that by 2027, 100% of people with TB should have access to a health and social benefits package.
- Globally in 2023, an estimated 0.96 million incident cases of TB were attributable to undernutrition, 0.75 million to alcohol use disorders, 0.70 million to smoking, 0.61 million to HIV infection, and 0.38 million to diabetes.
- From 2019 to 2023, WHO worked with high TB burden countries to ensure the inclusion of accountability mechanisms in national budget planning and pursuing assessment during high-level missions and joint TB programme reviews with engagement of civil society representatives, in line with WHO's multisectoral accountability framework on TB.
- The Global TB Report features a TB-SDG monitoring framework that focuses attention on 14 indicators that are associated with TB incidence. Monitoring of these indicators can be used to identify key influences on the TB.

TB FINANCE

- At the 2023 UN high-level meeting on TB, Member States committed to mobilizing at least US\$ 22 billion for TB prevention, diagnosis, treatment and care by 2027.
- There was a decline in global funding available on essential TB services from US\$ 6.8 billion in 2019 to US\$ 5.7 billion in 2023, which is only 26% of the global target of US\$ 22 billion.
- As in the previous 10 years, most of the spending on TB services in 2023 (80%) was from domestic sources. From 2019 to 2023, there was a decline (of US\$ 1.2 billion) in available funding from domestic sources and a very slight increase (of US\$ 0.1 billion) in funding provided by international donors. Most of the reduction in domestic funding is explained by reductions in domestic funding in Brazil, the Russian Federation, India, China and South Africa (BRICS).
- Financing for TB research at US\$ 1.0 billion in 2022 also continues to fall far short of the global target of US\$ 5 billion per year, constrained by the overall level of investment.

WHO's GLOBAL TUBERCULOSIS PROGRAMME together with WHO regional and country offices: develops policies, strategies and standards; supports the efforts of WHO Member States; measures progress towards TB targets and assesses national programme performance, financing and impact; promotes research; and facilitates partnerships, advocacy and communication.

More information: www.who.int/tb